PART 1

The Emergency Approach: Safety, Triage and Assessment

IN THIS PART

1. Describe the scene and personal safety steps taken before patient contact.
2. Explain how triage assigns priority using a generic five-level idea.
3. Describe the order of the primary survey and the check made at each step, including level of consciousness and a blood glucose check.
4. Describe how to gather a focused history and a set of vital signs, and recognize and report a worsening trend.
5. Describe a structured handover and the reassessment and documentation that go with it, within your scope of practice.

Every emergency starts the same way. Someone arrives at a scene they did not choose, takes in far more than they can process, and has a few seconds to decide what matters first. People who handle this well have a fixed order of thinking and follow it while their pulse is racing.

This Part gives you that order. It begins with the ground you are standing on, because a responder who becomes a casualty helps no one. It then moves to sorting, the quiet decision about who can wait and who cannot. After that comes the primary survey, a short sequence of checks that finds the problems most likely to kill a patient soonest. The Part closes with the slower work that follows: gathering a history, measuring vital signs, handing the patient over clearly, and checking again.

Read it as a map for the rest of the book. Parts 2 to 6 deal with particular problems, but each of them assumes you can arrive safely, sort, assess in order and communicate. Where a skill depends on your training level, the text says so. What you may do is set by your scope of practice and local protocol, not by this book.

1.1 Scene Safety, Personal Protection and Responder Wellbeing

Before you kneel beside a patient, stop and look at the scene once more. Check for traffic, spilled fluid, loose electrical equipment, unstable structures, animals, and any person who may still be a danger. Your first duty is to arrive able to help. An injured responder is a second patient and takes a rescuer away from the first.

WHAT IT IS. Scene safety is the habit of judging risk before contact. It has three parts: the environment, the people present, and your own protection. A few deliberate seconds at the edge of a scene cost far less than treating a second patient.

WHY IT MATTERS. Emergencies create their own hazards. A crash scene may have traffic still moving. A patient who is confused or frightened may strike out without meaning to. A spilled

fluid may be slippery or harmful to skin. Because you are focused on the patient, you are the person least likely to notice these things, so the check must be a routine step you take every time.

PERSONAL PROTECTION. Standard precautions are the baseline for every patient. They include hand hygiene, personal protective equipment, respiratory hygiene and cough etiquette, safe handling of needles and sharps, and careful cleaning and handling of equipment, textiles and the surrounding environment. The guiding rule is to use protective equipment whenever you expect possible contact with infectious material, such as blood or body fluids. For most emergency contacts that means gloves first, with eye protection, a mask or a gown added when splashes or sprays are likely. Your service, school or hospital sets the exact requirements, and you follow them.

PROCEDURE: MAKE THE SCENE SAFE

Purpose: to reach the patient without becoming a second casualty.
Scope: follows your training level and local protocol.

1. Stop at the edge of the scene and look in every direction before you move in.
2. Put on gloves and any other protection your role and the situation require.
3. Name each hazard aloud, even if you are alone, so you do not skip one.
4. Remove a hazard only if it is safe and within your role; otherwise keep clear and wait for help.
5. Count the patients, and ask bystanders to keep back and, if needed, to call for more help.
6. Approach, tell the patient who you are, and begin the primary survey (Entry 1.3).

> **⚠ SAFETY**
>
> Do not enter a scene you believe is unsafe. If violence, fire, unstable ground, live electrical equipment or traffic is present and you cannot control it, stay back and call for help.

STOP AND CALL FOR HELP IF a hazard remains, if you are unsure whether it has gone, or if the number of patients is more than you can manage.

The steps work because they put thinking before action. Naming hazards aloud turns a vague feeling of unease into a list you can act on, and counting patients early tells you how much help to request while you still have a free hand to request it.

RESPONDER WELLBEING. Emergency work asks you to see injury, fear and sometimes death, and it is normal for this to leave its mark. Do not treat strong reactions after a hard call as a personal failing. Eat, drink and rest when you can, tell a trusted colleague or supervisor when something stays with you, and ask whether your service or school offers a debrief or peer support. Seeking that help is part of staying safe to work.

NURSING AND RESPONDER PRIORITIES. Check the scene before touching the patient, protect yourself with standard precautions, call for more help early if the scene or the number of patients is beyond your resources, and look after your own wellbeing as part of the job.

See also: Entry 1.3 (primary survey); Entry 1.5 (communication and handover).

1.2 Triage and Acuity

Triage is not a judgment about who deserves care. It is a way of deciding who cannot wait. A patient who walks in talking, holding a swollen ankle, can sit for a while. A patient who looks calm but is breathing far too fast cannot, and the person at the desk has only moments to notice. This entry gives you a short, repeatable way to sort patients so that the quiet emergency is not missed behind the loud inconvenience.

WHAT IT IS. Triage is the process of sorting patients by urgency so that limited people, space and equipment reach the sickest first. The word comes from the French for sorting. In a US emergency department (ED), triage is usually done by a specially trained nurse soon after the patient arrives. At a scene with many casualties, the same logic is applied quickly and with little equipment, under local protocol. Different places use different tools, and local protocol decides which one you use.

WHY IT MATTERS. Emergency departments, ambulances and scenes are often not staffed for every patient at once. When demand is higher than capacity, the order in which patients are seen is itself a treatment decision. A good sorting method protects the patient who does not complain, such as the older adult with a vague story who may be seriously ill, from being passed over in favor of someone who is louder or more distressed but more stable.

A FIVE-LEVEL IDEA. Many emergency departments in the United States sort patients into five levels of acuity, where level one is the most urgent and level five the least. You do not need the details of any particular published tool to understand how the principle works. It rests on a small number of ordered questions.

REMEMBER

Sort by danger before convenience. Look first for the patient who may die soon, then for the one who must not wait, then count what each of the others will need, and let the vital signs have the last word.

Three questions, asked in order, do the sorting. The first finds the patient who needs action at once, for example from an obstructed airway or a stopped heart. The second finds the patient who should not wait even though nothing must happen in the next few seconds. Everyone else is sorted by how many different kinds of resources they will probably need, such as laboratory tests, imaging, intravenous treatment or a procedure. Simple care such as an examination, a dressing, a splint or a medicine taken by mouth does not count. The last step is to check the vital signs against the level you chose and consider moving the patient up if they are in a danger range (Entry 1.4 gives typical resting values).

Figure 1 shows the sequence.

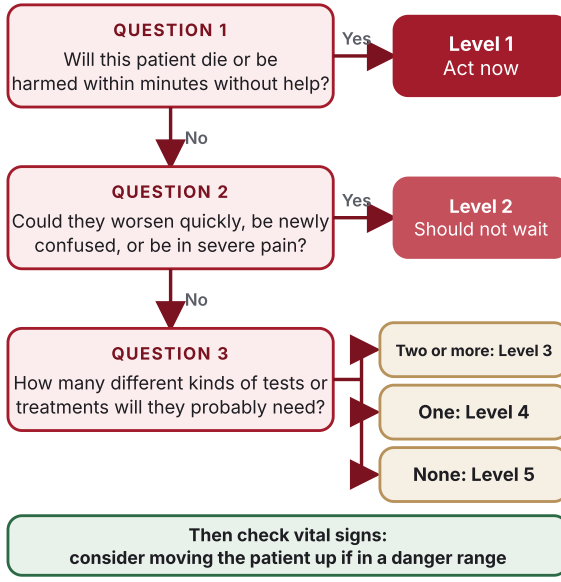


Figure 1. A generic five-level triage sequence, in which the first two questions ask about danger, not paperwork.

Five levels of acuity, in plain terms

Level	Plain description	Typical response
1	Needs immediate life-saving action to stay alive	Treated immediately
2	Could worsen quickly, newly confused, or in severe pain	Should not wait; seen as soon as possible
3	Stable, but expected to need two or more kinds of resources (for example, a blood test and imaging)	Waits in order, rechecked
4	Stable, expected to need one kind of resource	Waits in order, rechecked
5	Stable, expected to need no resources beyond an examination and simple care	Waits in order, rechecked

A level is a starting point, not a verdict. Patients move between levels as you watch them. A person given level four on arrival who becomes short of breath in the waiting room must be reassessed at once and moved to a higher level.

WHAT TRIAGE IS NOT. Triage does not diagnose. It does not say what is wrong, only how soon someone must be seen. Two patients with the same level are often seen in the order they arrived, unless one gets worse. A patient who waits longer is one whose condition allows waiting. Say this to patients and families in plain words, because waiting is easier when people understand the reason for it.

COMMON PITFALLS. Four errors appear again and again. The first is anchoring on the loudest complaint and missing the quiet one. The second is trusting a calm appearance, because patients who are very ill may be quiet, tired or confused, not agitated. The third is using one set of vital signs as if it were permanent. The

fourth is letting the label stick: a patient already sorted to a low level can be overlooked for a long time unless someone is responsible for rechecking. Build a habit of glancing at the waiting area each time you pass it.

RED FLAGS

Move the patient up a level and call for help for: difficulty speaking in full sentences, a gray or blue tint to the lips, new confusion or drowsiness, a patient who stops responding, severe bleeding, or a sudden change in how the patient looks compared with a few minutes ago.

SCOPE NOTE

Assigning a triage level is often a nursing role in an emergency department and is done under local protocol. Students and other responders observe or assist under supervision. If you notice a red flag in the waiting area, tell the responsible nurse at once; do not wait to be asked.

NURSING AND RESPONDER PRIORITIES. Sort by danger first and by resources second, recheck often, reassure waiting patients with an honest explanation, and escalate at once when a patient looks worse than their label.

See also: Entry 1.3 (primary survey); Entry 1.4 (vital signs); Entry 6.1 (children and acuity).

1.3 The Primary Survey

The primary survey is the short sequence of checks that finds the problems most likely to kill a patient soonest. It runs in a fixed order, and you treat a life-threatening problem the moment you find

it, before moving on. The order is Airway, Breathing, Circulation, Disability and Exposure, usually written ABCDE. This order is the spine of the whole book, and every later Part returns to it.

WHAT IT IS. The primary survey is a rapid, whole-patient look for immediate threats to life. It does not diagnose and it does not replace a full examination. Its aim is to keep the patient alive and improve their condition, which buys time for further treatment and for working out what is wrong. The same approach applies to every critically ill or deteriorating patient, whatever the cause.

WHY THE ORDER MATTERS. The letters follow how quickly each failure kills. A blocked airway stops oxygen reaching the lungs within moments, so nothing that comes later can help until the airway is open. Breathing comes next because an open airway is useless without air moving. Circulation follows because oxygen must be carried to the brain and heart. Disability, which means checking how the brain is working, comes after those, and Exposure, which means looking at the rest of the body, comes last. Severe bleeding is the one exception that rearranges the sequence. When blood loss is heavy and visible, many trauma protocols direct you to control it first, because there is little point in checking the airway of a patient who is bleeding to death. Entry 4.1 explains how bleeding is brought forward in trauma.

> **REMEMBER**
>
> Find it, fix it, check again. After any treatment, wait a short while before judging its effect, then repeat the survey.

THE FIRST LOOK. Before the letters, take a rapid first look at the patient in general. If the patient is awake, ask a simple question such as “How are you?” If they appear unconscious, speak to them loudly. If you can rule out a neck injury, tap the shoulders gently as you ask whether they are all right; if a neck injury is possible,

do not shake the patient and keep the head and neck still. A patient who answers in a normal voice has an open airway, is breathing and is probably getting enough blood to the brain for the moment. A patient who speaks only in short sentences may have a breathing problem. A patient who does not respond is critically ill until shown otherwise. This look takes about 30 seconds and often tells you at once that the patient needs urgent help. If the patient is unresponsive and not breathing normally, call for help and start cardiopulmonary resuscitation (CPR) as set out in Entry 3.4. Occasional gasps are not normal breathing.

AIRWAY. Ask whether air can get in and out. Talking means the airway is open. Noisy breathing is a warning, because a partly blocked airway is often noisy, and a high-pitched sound such as stridor suggests a significant partial blockage. Complete blockage may give no sound at all, with the chest and abdomen moving in opposite directions as the patient strains. Clear and open the airway with the simple measures in Entry 2.2, and call for help at once.

BREATHING. Look, listen and feel. Count the breathing rate (respiratory rate, RR) carefully, for as long as your protocol directs, and watch for effort: sweating, use of the neck and shoulder muscles, belly breathing, or a gray or blue tint to the lips. A high or rising rate is a warning that the patient may deteriorate suddenly. Note how deeply the patient breathes and whether both sides of the chest move equally. If you have a pulse oximeter, record the reading, and remember what it does not tell you, as Entry 1.4 explains.

CIRCULATION. Look at the skin of the hands: is it pink, pale, blue or mottled? Feel whether the hands are warm or cool. Check the capillary refill time by pressing on a fingertip, held level with the heart, for 5 seconds, then timing how long the color takes to

return. In a healthy adult it is usually under 2 seconds, and a longer time suggests poor blood flow to the skin, though a cold room or cold hands can also lengthen it. Feel a pulse and note whether it is fast, slow, weak, strong or irregular. A bounding pulse and a barely palpable one both matter. Look for bleeding you can see, and think about where blood might be hiding, in the chest, abdomen or pelvis. Entry 3.5 explains shock in full.

DISABILITY. This step checks how the brain is working. Begin with the quickest tool, the AVPU scale. Is the patient Alert? Do they respond only to Voice? Only to Pain? Are they Unresponsive? Some services add a letter for new confusion, so that the scale reads A, C, V, P, U, and new confusion in a previously normal patient is treated as a serious sign. Where your protocol uses it, the Glasgow Coma Scale gives a fuller picture. It scores three responses: eye opening, verbal response and motor response. The three scores are added to give a total from 3 to 15, where 15 is the highest score. Lower scores mean greater impairment, and the trend over time is important in showing when new action is needed. Check the pupils for size, equality and reaction to light, and check the blood glucose, because low blood glucose can cause confusion or loss of consciousness and is treatable. Entry 5.1 covers its causes and management.

EXPOSURE. To examine a patient properly you may need to expose the body. Do it with respect for dignity, with screens or blankets where possible, and take care that the patient does not lose heat. Look for wounds, rashes, bruising, medical alert jewelry, and anything that was hidden by clothing.

PROCEDURE: THE PRIMARY SURVEY

Purpose: to find and treat immediate threats to life in the order they kill. Scope: follows your training level and local protocol.

1. Make the scene safe and put on protection (Entry 1.1).

2. Take the first look: speak to the patient and watch the response for about 30 seconds; if heavy bleeding is visible, your protocol may direct you to control it first (Entry 4.1).
3. If the patient is unresponsive and not breathing normally, call for help and begin CPR.
4. Airway: confirm the patient can speak or breathe freely; open and clear it if not.
5. Breathing: count the rate, look at the effort and chest movement, and record the oxygen saturation if a device is available.
6. Circulation: check skin color and warmth, capillary refill and pulse; control any severe bleeding.
7. Disability: check AVPU or Glasgow Coma Scale, the pupils and the blood glucose.
8. Exposure: expose as needed, protect dignity and warmth, and look for injuries.
9. Treat each life-threatening problem as you find it, call for help, and repeat the survey after every intervention.

> ⚠ **SAFETY**
>
> Treat the neck (cervical spine) with care if there is any chance of neck injury, and do not move an injured patient unless the scene is unsafe or your protocol directs a move to give life-saving care, and then keep the head, neck and body in line as far as you can.

STOP AND CALL FOR HELP IF any step finds a threat to life that you cannot correct, or if the patient does not improve after you treat it.

Each step in this procedure answers a single question, and the answers build on one another. When you finish the lettered checks, you can say whether the patient is stable, unstable, or deteriorating, and you can state in a sentence what you did and how the patient responded. That sentence becomes the core of your handover in Entry 1.5.

CHECKING BLOOD GLUCOSE. The blood glucose check is one step inside Disability, and it is a skill only where your role and protocol allow it. A meter reads a small drop of blood from a fingertip. Use the device exactly as its maker directs, wear gloves, use a new single-use lancet each time, and place the used lancet in a sharps container at once. For many people with diabetes, a reading below 70 milligrams per deciliter (3.9 millimoles per liter) is low, though the number that counts as low can differ from person to person. The number is a prompt to act under your protocol, not a diagnosis. Entry 5.1 describes the response. If your role does not include glucose testing, report that you could not check it and tell the responsible clinician.

Figure 2 shows the loop.

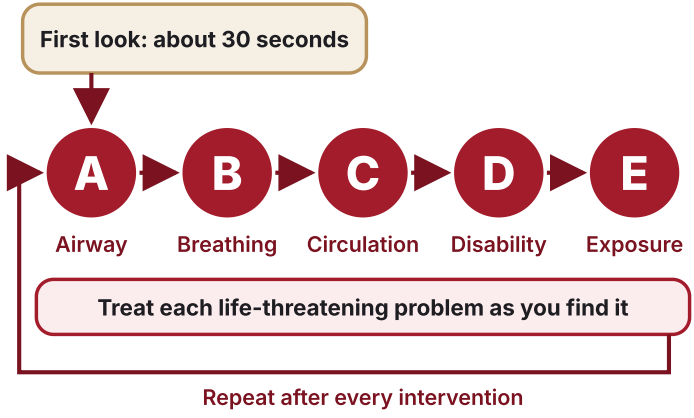


Figure 2. The primary survey as a loop, with treatment after each check and a repeat of the whole loop after every intervention.

NURSING AND RESPONDER PRIORITIES. Complete the first look quickly, work through the letters in order, treat what you find before moving on, call for help early, and repeat the sequence whenever the patient changes or after any intervention.

RED FLAGS

Unresponsiveness, absent or abnormal breathing, noisy or absent airflow, blue or gray lips, a very fast or very slow pulse, cold mottled skin, heavy bleeding, a falling level of consciousness, or a new confusion in a patient who was normal a short time ago.

See also: Entry 1.4 (vital signs); Entry 2.2 (airway); Entry 3.5 (shock); Entry 4.1 (trauma); Entry 5.1 (altered mental status).

1.4 History, Vital Signs and the Secondary Survey

Once the primary survey has found and treated immediate threats, you have time to learn more. The secondary survey is a slower, systematic look from head to toe combined with a focused history. Its job is to catch what the primary survey was not designed to find, and to give the receiving team a clear picture. It never replaces the primary survey, and you return to the primary survey whenever the patient worsens.

WHAT IT IS. The secondary survey has two strands that run together. One is the history, which asks the patient, family, bystanders and records what happened. The other is the examination, which includes a full set of vital signs and a head-to-toe check. In an unstable patient the secondary survey is usually shortened or postponed so that treatment and transport are not delayed.

GATHERING A HISTORY. A short, structured set of questions keeps the history focused. Ask what happened and when it started. Ask about the main symptom: where it is, how bad it is and what makes it better or worse. Ask about allergies, current medicines, past illnesses, the last meal or drink, and the events leading up to the problem. These questions are often remembered with a mnemonic, and your school or service will teach one. Use the mnemonic you are taught and cover the same ground in the same order every time, so that nothing important, such as an allergy or a blood-thinning medicine, is forgotten. If the patient cannot talk, turn to family, bystanders, medical alert jewelry or records, and note who gave each piece of information.

VITAL SIGNS. Vital signs are measurements of essential body functions: heart rate, breathing rate, blood pressure and temperature, usually with oxygen saturation added. Normal values change with age, sex, weight, fitness and overall health, so the ranges below describe a healthy adult at rest and should be read as a guide, not a rule.

Typical resting ranges for a healthy adult

Measure	Typical range
Pulse (heart rate)	60 to 100 beats per minute
Breathing rate	12 to 18 breaths per minute
Blood pressure	between 90/60 and 120/80 mm Hg
Temperature	97.7 to 99.1 degrees F (36.5 to 37.3 degrees C)

A reading outside its range may be abnormal for that patient. Report it, and treat a large departure, or any departure in a patient who looks unwell, as urgent.

Treat the trend as more informative than any one reading. A breathing rate that climbs from 18 to 28 over 20 minutes tells you more than a single count, and a breathing rate above about 25 breaths per minute is a marker of illness and a warning that the patient may deteriorate suddenly, so report it to your supervisor or the responsible clinician at once. Blood pressure can stay normal for a time even in shock, because the body tightens its blood vessels to compensate, so a normal reading does not by itself make a patient safe.

OXYGEN SATURATION. A pulse oximeter clips on a finger and estimates the percentage of hemoglobin carrying oxygen. It is useful, but it has limits that you must know. It does not measure how well a patient is breathing out carbon dioxide, so a patient can

have a normal reading while still ventilating poorly. Readings can also be less accurate with poor circulation, cold hands, some nail polish colors, motion and differences in skin pigmentation, and the device can be misled by carbon monoxide poisoning. Read the number together with the patient's color, effort and mental state. A reassuring number in a patient who looks ill should not reassure you.

Figure 3 shows the places to check.

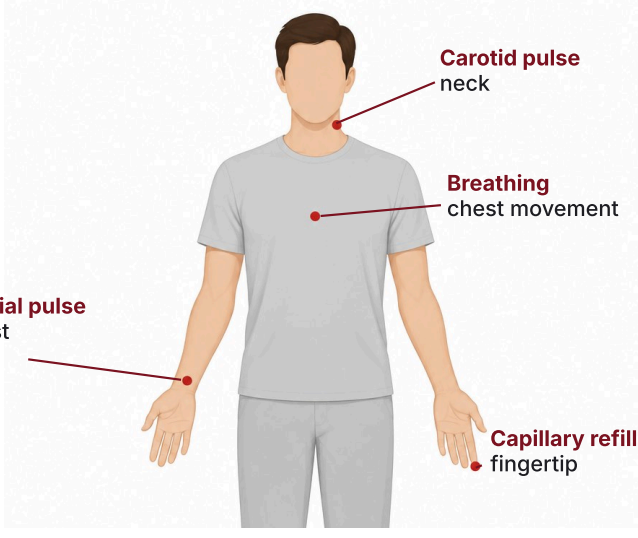


Figure 3. Where to check pulse, breathing and skin perfusion on an adult, in the same places each time.

HEAD-TO-TOE CHECK. Work from the head down: scalp and face, eyes, ears, nose and mouth, neck, chest, abdomen, pelvis, back, arms and legs. Look, then feel gently, and ask about pain. Note wounds, deformity, swelling, bruising and anything that looks wrong or smells wrong. Record the time of each set of observations.

SCOPE NOTE

Taking vital signs is a core skill for nursing, paramedic and emergency medical technician (EMT) students, but interpreting patterns and acting on them is done with your supervisor and under local protocol. Always tell a qualified clinician about an abnormal or worsening finding at once.

NURSING AND RESPONDER PRIORITIES. Return to the primary survey if the patient deteriorates, take a complete set of vital signs and record the time, trust trends over single numbers, ask structured history questions the same way each time, and treat reassuring numbers with caution when the patient looks unwell.

See also: Entry 1.3 (primary survey); Entry 5.4 (sepsis); Entry 6.1 (children's values).

1.5 Communication, Consent, Scope of Practice, Handover, Reassessment and Documentation

A patient cared for well by three people is easily harmed when the three do not talk to each other. This last entry covers the work that holds a response together: speaking with the patient, obtaining consent, staying within your scope, passing information on clearly, checking again, and writing down what you saw and did.

SPEAKING WITH THE PATIENT AND CONSENT. Introduce yourself, say what you are about to do and why, and ask permission before you touch. A patient who has capacity, meaning the ability to understand the information, see how it applies to their own situation, weigh the options and state a choice, may accept or refuse care, and you respect that choice, tell a clinician and record it. In a true emergency, when a patient cannot take part in deci-

sions and no one who can decide for them is available, treatment may start without prior consent, and the patient is told as soon as possible. The rules differ between states, services and roles, so follow your local protocol and the guidance of your supervisor.

SCOPE OF PRACTICE. Your scope of practice is the set of tasks you are legally authorized and trained to perform. It depends on your license or certification, your education, your role and the setting, and students work under the rules of their program and their supervisor. Do only what it allows, say plainly when a task is beyond it, and ask your supervisor before trying anything you have not been taught and cleared to do.

HANDOVER. Handover is the structured passing of a patient's information to the next team. One widely used format has four parts: Situation, Background, Assessment and Recommendation. Situation says who the patient is and what the main problem is. Background gives the history, medicines, allergies and what led up to this. Assessment gives your findings in the order of the primary survey, the vital signs and what you did. Recommendation states what you think should happen next or what you need from the receiver. A good handover is short, follows a fixed order, and labels a guess as a guess.

Figure 4 shows the four parts.

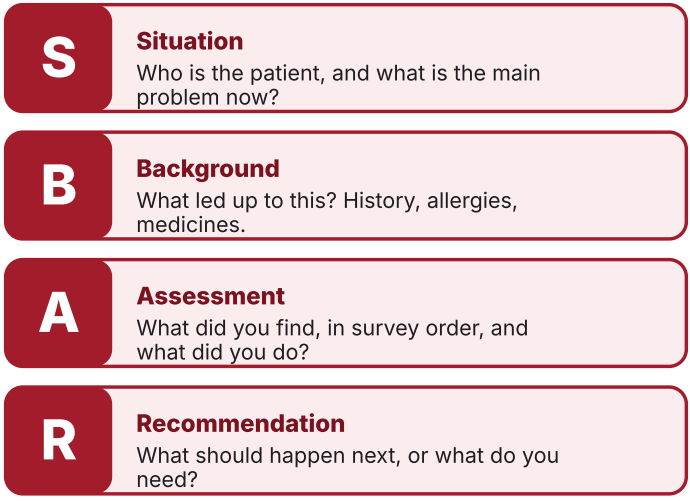


Figure 4. A four-part handover in which each part answers one question for the receiver.

REASSESSMENT. A patient who was stable a moment ago may not be stable now. Recheck the primary survey and vital signs at intervals set by the patient's condition and your protocol, after every treatment, and before and after any move.

DOCUMENTATION. Write down what you found, what you did, when you did it and how the patient responded, using the times you recorded. Record facts and observations, not opinions, and keep the record accurate and legible. A clear record lets the next team see what has already been done and when.

NURSING AND RESPONDER PRIORITIES. Explain what you are doing, obtain consent or follow the emergency rules for those who cannot give it, hand over in a fixed order, recheck often, and record accurately and promptly.

See also: Entry 1.3 (primary survey); Entry 1.4 (vital signs).

Illustrative Case

ILLUSTRATIVE CASE: NOT A REAL PATIENT

Priya was a second-year nursing student volunteering at a school sports day. She was carrying a water cooler across the field when a spectator in the stands, a man in his sixties, slid sideways on the bench. People nearby stood up and shouted. Priya stopped at the foot of the stands and looked before climbing. The bleachers were crowded and the step was wet from a spilled drink, so she slowed down, pulled on gloves from her kit bag, and asked a nearby parent to find the event first-aid lead and bring the automated external defibrillator.

She reached the man and said her name. His eyes opened when she spoke loudly, and he mumbled a few words. His breathing was quick and shallow, and he could say only two or three words at a time. His lips looked pale. She asked the parent to call emergency services and to give the stand number. Then she began the survey she had practiced in class.

His airway was open, because he was making sound. She counted his breathing carefully and found it fast. His fingertips were cool and the color took longer than usual to return after she pressed. His pulse was quick and weak. He responded to voice but not to questions, so she noted that he was not fully alert. When the first-aid lead arrived, Priya gave the handover she had rehearsed: who he was, what she had seen, her findings in order, and that he seemed to be getting worse. The lead took over and asked her to stay, record the times and keep checking his breathing until the ambulance came.

Care Priorities

CARE PRIORITIES

When two things compete, your own safety comes before the patient's need, and the patient who cannot wait comes before the patient who complains most. Treating a threat comes before measuring it: a pulse oximeter reading can wait, an obstructed airway cannot. Calling for help comes before finishing the history, the head-to-toe check or the paperwork. A plan is only as good as the next check, so put a time on every observation and decide before you leave the bedside when you will look again. If a task is outside your scope, say so at once and stay with the patient until someone who can do it arrives.

Review Questions

1. What should you do first when you arrive at an emergency scene?
   - **A)** Begin the primary survey at once.
   - **B)** Check the scene for hazards and protect yourself.
   - **C)** Take a full set of vital signs.
   - **D)** Record the time on the patient's chart.

2. Triage sorts patients mainly by which factor?
   - **A)** The order in which they arrived.
   - **B)** How loudly they complain.
   - **C)** How long they are expected to stay.
   - **D)** How soon they must be seen to stay safe.

3 A patient sorted to a low level of acuity becomes short of breath in the waiting room. What is the best response?

A) Reassess, raise the level and call for help.
B) Leave the level unchanged, since a nurse assigned it.
C) Ask the patient to wait quietly for their turn.
D) Take only their temperature.

4 A patient has a normal blood pressure, cool pale skin and fast breathing. What is the best interpretation?

A) The patient is stable because the blood pressure is normal.
B) The skin color is caused by cold hands and can be ignored.
C) The patient may still be seriously ill, because blood pressure can stay normal for a time in shock.
D) The breathing rate does not matter when the blood pressure is normal.

5 A patient responds only when you speak loudly to them. On the AVPU scale, which level describes this?

A) Alert.
B) Pain.
C) Unresponsive.
D) Voice.

6 Which of these is a limitation of a pulse oximeter?

A) It does not show how well a patient is breathing out carbon dioxide.
B) It cannot be used on a finger.
C) It can only be used when the patient is awake.
D) It replaces the need to count the breathing rate.

7 In a handover that uses Situation, Background, Assessment and Recommendation, where do the primary survey findings belong?

A) Situation.
B) Background.
C) Assessment.
D) Recommendation.

8 Short case. You find an older adult slumped on a bench in a park, breathing fast and responding only to voice. List, in order, the first four things you would do.